

Preeclampsia Prevention Chart Checklist

1 / Patient history and front-office routing

Patient name: _____ DOB: _____ MRN: _____

Visit date: _____ Visit type: _____ GA: _____ weeks _____ days

Place patient label here if preferred. Repeat identifiers on every page.

A. FRONT OFFICE - distribute and route; do not make clinical decisions

☐ Patient label / identifiers checked ☐ Blank form given ☐ Interpreter requested

Language / assistance: _____ Staff initials: _____ Time: _____

If urgent symptoms are reported, alert the clinical team immediately using the office emergency process. Do not wait for form completion or independently reassure the patient.

☐ No concern reported ☐ Concern routed to clinical team ☐ Unable to complete

Clinical staff notified: _____ Date / time: _____

B. PATIENT - tell staff immediately if you have concerning symptoms

Severe headache that will not go away, vision changes, severe belly pain, chest pain, or trouble breathing need medical care right away. This list is not complete. If outside the office, call your care team; if unreachable, go to the emergency department. Call 911 for a life-threatening emergency.

C. PATIENT - mark Yes, No, or Not sure; clinician will confirm

Check ONE answer per row; blank = not completed.

	Yes	No	Not sure
Preeclampsia in a previous pregnancy?	☐	☐	☐
High blood pressure before this pregnancy?	☐	☐	☐
Type 1 or type 2 diabetes before this pregnancy?	☐	☐	☐
Kidney disease?	☐	☐	☐
Lupus or antiphospholipid syndrome?	☐	☐	☐
Pregnant with twins or more babies?	☐	☐	☐
Is this your first pregnancy?	☐	☐	☐
Mother or sister had preeclampsia?	☐	☐	☐
This pregnancy conceived using IVF?	☐	☐	☐
Over 10 years since your last pregnancy?	☐	☐	☐
Prior pregnancy problem (for example, a very small baby)?	☐	☐	☐
You were born at a low birth weight / small for gestational age?	☐	☐	☐
Past aspirin / anti-inflammatory medicine reaction?	☐	☐	☐
Stomach ulcers, bleeding problems, or serious liver disease?	☐	☐	☐

Are you taking aspirin now? Yes / No / Not sure Dose: _____ Since: _____

Other medicines / concerns: _____

Patient / assisting person initials (optional): _____ Clinician review pending.

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2 / Doctor or APP - risk confirmation and safety review

Patient name: _____ DOB: _____ MRN: _____

Visit date: _____ Visit type: _____ GA: _____ weeks _____ days

Place patient label here if preferred. Repeat identifiers on every page.

D. CONFIRM RISK - patient answers alone do not establish eligibility

Check ONE answer per row; blank = not completed.

	Present	Absent	Unknown
History of preeclampsia (especially with adverse outcome)	☐	☐	☐
Multifetal gestation	☐	☐	☐
Chronic hypertension	☐	☐	☐
Pregestational type 1 or type 2 diabetes	☐	☐	☐
Kidney disease	☐	☐	☐
Autoimmune disease (SLE / antiphospholipid syndrome)	☐	☐	☐

High-risk factors confirmed: _____ Any unknowns / plan: _____

Check ONE answer per row; blank = not completed.

	Present	Absent	Unknown
Nulliparity	☐	☐	☐
Prepregnancy BMI >30 (BMI: _____)	☐	☐	☐
Age at delivery >=35 years (age: _____)	☐	☐	☐
Family history: mother or sister with preeclampsia	☐	☐	☐
In vitro fertilization	☐	☐	☐
Personal history category*	☐	☐	☐
Self-identified Black race (proxy for racism, not biology)	☐	☐	☐
Lower income (clinical discussion; do not infer from insurance alone)	☐	☐	☐

*Personal history: low birth weight / SGA, previous adverse pregnancy outcome, or >10-year pregnancy interval.

Count this category once, even if multiple examples apply.

Distinct moderate-risk factors: _____ Supporting history: _____

E. CLINICIAN INTERPRETATION - select one and explain exceptions

- ☐ >=1 high-risk factor ☐ >=2 moderate-risk factors
☐ Black identity or lower income alone: consider aspirin ☐ Other / individualized
☐ No indication identified ☐ Assessment incomplete: clarification needed

ACOG/SMFM permits consideration with Black race (a proxy for underlying racism) or lower income alone. Social and structural inequities drive this risk; race is not a biological cause.

F. SAFETY REVIEW - complete before recommending aspirin

- ☐ Medication / allergy list reviewed ☐ Contraindications assessed
☐ Aspirin / salicylate / NSAID hypersensitivity ☐ Nasal polyps / aspirin bronchospasm
☐ GI bleeding / active ulcer / other bleeding ☐ Severe liver dysfunction
☐ No concern identified ☐ Concern present ☐ Uncertain: resolve before prescribing

Details / interaction review / individualized risk: _____

Reviewed by (doctor / APP): _____ Initials: _____ Date: _____

Guidance: ACOG/SMFM aspirin advisory (2021); ACOG Committee Opinion 743 (2018). Review gestational timing and sign the final clinical plan on page 3.

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3 / Doctor or APP plan, follow-up, and chart filing

Patient name: _____ DOB: _____ MRN: _____

Visit date: _____ Visit type: _____ GA: _____ weeks _____ days

Place patient label here if preferred. Repeat identifiers on every page.

G. COUNSELING - mark completed, N/A with reason, or not completed

Check ONE answer per row; blank = not completed.

	Done	N/A	Not done
Personal risk and reason for recommendation discussed	☐	☐	☐
Benefits and limits: risk reduced, not eliminated	☐	☐	☐
Safety, side effects, medication questions reviewed	☐	☐	☐
Dose, start date, duration and missed-dose instructions reviewed	☐	☐	☐
Access, cost, adherence barriers and preferences discussed	☐	☐	☐
Warning signs and contact / follow-up plan reviewed	☐	☐	☐

☐ Teach-back completed ☐ Education handout provided ☐ Interpreter used

N/A reasons / unresolved questions: _____

H. ASPIRIN DECISION - clinician selects ONE primary status

☐ Initiated today ☐ Continued ☐ Not indicated ☐ Patient declined
☐ Contraindicated ☐ Held temporarily ☐ Pending clarification ☐ Planned start

Indication / reason for decision or exception: _____

Dose: _____ mg by mouth daily Actual start date (if known): _____

GA at actual start: _____ weeks _____ days / unknown Planned start date: _____

Planned start GA: _____ weeks _____ days Duration / stop plan: _____

US guidance: 81 mg daily; initiate at 12-28 weeks, ideally before 16 weeks; continue until delivery unless individualized. Before 12 weeks, document a future plan. After 28 weeks or for a different dose, document clinical reasoning. This form is not a standing medication order.

☐ Order / medication list updated when indicated ☐ Instructions given to patient

I. FOLLOW-UP AND SIGN-OFF - doctor / APP or clinical care team

Next review / visit: _____ BP plan: _____

Office / after-hours contact given: _____

At follow-up: taking as directed? Yes / No / Unknown / N/A Barrier: _____

Follow-up action / responsible clinician: _____

Doctor / APP name & role: _____ Signature: _____

Decision date / time: _____ GA at decision: _____ weeks _____ days

J. FRONT OFFICE / RECORDS - file after clinician completion

☐ All 3 pages identified ☐ Signed clinical plan present ☐ Filed in correct chart

Filed by initials: _____ Date: _____ Clinical team notified if incomplete: _____

Keep completed pages in the clinical chart. Do not send names, DOBs, MRNs, or signatures to the researcher. Use only the approved de-identified extraction process. Paper adaptation is not research consent or proof of enrollment.